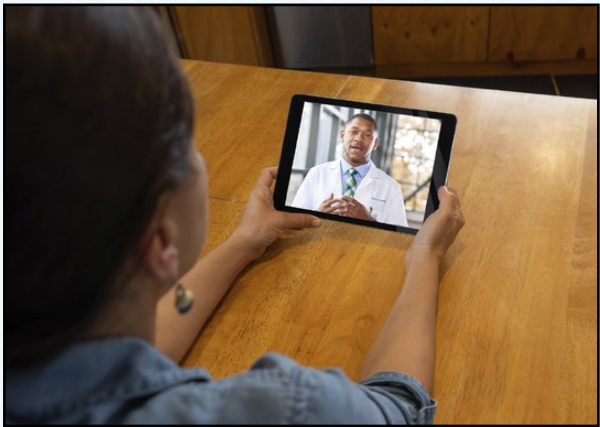




When to Provide Virtual Care

Clinical Case Quiz



A 4 ½ -year-old boy with a history of asthma presents via video visit with 2 days of fever (Tmax 103°F) and abrupt onset of “severe” productive cough with intermittent shortness of breath and wheezing, only somewhat relieved by albuterol. Mother reports a home SpO2 of 95%.

On video exam: Patient is awake and alert without cyanosis but has mild tachypnea and intercostal retractions. He has a frequent, non-barky cough. When you ask the child to sing the ABCs, he needs to take occasional pauses to catch his breath every few letters.

- What is the most appropriate next step?
- a. Empirically start amoxicillin for presumptive “typical” bacterial pneumonia.
 - b. Empirically start azithromycin for presumptive atypical pneumonia.
 - c. Order a STAT CXR to help rule in or rule out pneumonia.
 - d. Start oral prednisolone and instruct the patient to use albuterol q4-6 hours.
 - e. Recommend a same-day in-person appointment for further evaluation.

The answer is e.

Discussion

Telemedicine has expanded worldwide, and clinicians are becoming more comfortable with virtual treatment options. It is important, however, to understand that in certain scenarios, an in-person evaluation is still the optimal choice for the patient.

When the differential diagnosis is broad, and the patient has acutely concerning symptoms (such as shortness of breath), it’s best to recommend an in-person evaluation (**Answer E**) to obtain prompt and accurate vital signs, perform a physical examination to help narrow the differential diagnosis, and to determine the best potential workup and treatment options.

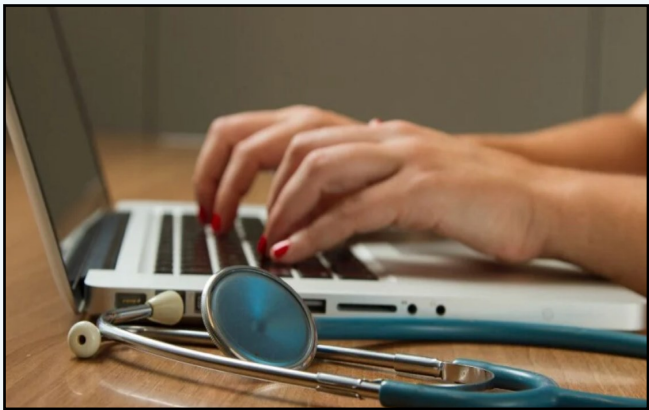
Answers A & B: It is **NOT** the standard of care to empirically treat bacterial pneumonia during pediatric virtual visits. Children may have viral pneumonia vs. “typical” bacterial pneumonia vs. atypical pneumonia vs. viral URIs with asthma exacerbation, bronchiolitis, influenza, etc. When considering pneumonia, children should be physically examined. Empirically starting antibiotics may not help the patient, and the wrong antibiotic may further delay care when the patient does not improve.

Answer C: CXR findings often lag behind clinical findings, so a negative CXR does not necessarily rule out pneumonia. This may also lead to a delay in care, since the patient is not guaranteed to obtain the CXR within a reasonable timeframe, and you will not be notified of the results until a radiologist reads it. Ordering a CXR virtually without a physical exam carries clinical and medico-legal risks.

Answer D: Although asthma exacerbation is possible, this patient should be seen and examined to determine an accurate diagnosis, especially given the shortness of breath.



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